

Market Intelligence

Compiled 2026-09-01 · method: VoC market-research protocol (parallel-stream sourcing, every claim linked, weaknesses mandatory) · for the Doronichev / Garber pitch

Fourteen top players in health-, wellbeing- and development-oriented mobile apps and games: how they measure results, what research they stand on and where it was done, how hypotheses transferred across markets, what the build cost, who funded them — and how the failures failed, including who got sued. Ends with the full pre-mortem risk table for Content Therapy.

TABLE 1 Player intelligence — 14 companies

Sorted roughly from most-regulated to pure-consumer. Every cell links to a primary or high-quality source.

PLAYER	HOW THEY MEASURE RESULTS	RESEARCH BASE → MARKET TRANSFER	MONEY & PARTNERS	FATE / DOCUMENTED WEAKNESS
EndeavorRx Akili · Rx video game, pediatric ADHD · FAILED COMMERCIALY	TOVA attention index (objective, computerized), ADHD-RS, CGI-I — pivotal STARS-ADHD RCT n=348 in <i>Lancet Digital Health</i> [FDA]	UCSF Gazzaley lab (NeuroRacer) → ~20 US trial sites incl. Duke → US-only Rx market; never scaled abroad; 2023 pivot to OTC after payers refused [Fierce]	>\$230M private (Neuberger, Polaris, Temasek, Shionogi) + \$1B SPAC; PureTech-founded [BW]	FDA clearance + Lancet RCT ≠ revenue: 2 layoff rounds, prescription model abandoned, sold for \$34M (−97% vs SPAC) [MedTech Dive]
Pear Therapeutics reSET / reSET-O / Somryst · PDT platform · BANKRUPT	Clinical endpoints per product (abstinence, ISI for Somryst 9-wk CBT-I) — 3 FDA clearances [FDA K191716]	US trials → US prescription + payer market that never materialized; ~half of prescriptions never reimbursed [STAT]	~\$400M+ private, \$1.6B SPAC (Dec 2021)	Chapter 11 Apr 2023; all assets auctioned for ~\$6M — regulatory clearance carried ~0 enterprise value without payment rails [Fierce]

Sleepio / Daylight Big Health · dCBT-I + anxiety · TRANSFER SUCCESS	SCI, ISI, sleep diaries; GAD-7 (Daylight); 13+ RCTs; NHS real-world n=7,000+ [Wiki/trials]	The canonical route: Oxford (Espie) RCTs → first-ever NICE guidance for a DTx (2022) → NHS Scotland national rollout → US employers/PBMs (Evernorth, CVS) → FDA 510(k) only in 2024 [NICE] [STAT]	~\$127M+ (SoftBank Vision Fund 2 led \$75M Series C) [BW]	Alive but monetization lagged evidence by a decade; nearly all trials authored by co-founder (disclosed COI); Trustpilot billing/rigidity complaints [Trustpilot]
Wysa AI chatbot + coach · NICHE-STABLE	PHQ-9 / GAD-7 in trials + deployments; registered US trial NCT05533190; real-world JMIR analyses [CT.gov]	Rare India → global path: Bangalore build → UK NHS waitlist support → US FDA Breakthrough designation (May 2022); anonymity-beats-stigma hypothesis proved portable [BW]	~\$30M; HealthQuad + British International Investment (UK state DFI) [TC]	2018 BBC-reported failures on abuse/self-harm disclosures; evidence heavy on engagement data, light on completed large RCTs [analysis]
Woebot CBT chatbot · CONSUMER EXIT	Peer-reviewed RCTs (PHQ-9 etc.), FDA Breakthrough for postpartum product	Stanford-born, US consumer + research; dropped FDA application 2023; consumer app closed Jun 30, 2025 — pivoted to enterprise licensing [STAT]	~\$114M raised	Squeezed both ways: FDA too slow & costly, LLMs reset user expectations — the safest actor was the one the structure punished
Kooth anonymous youth support · PROFITABLE PLC	Goal-based & session-level outcomes (built for anonymous drop-in care where PHQ-9 fits poorly); peer-reviewed economic evaluation [ScienceDirect]	Two decades of UK NHS service data → US government contracts: California "Soluna" ~\$188M, Pennsylvania — sells to states, not app stores [KFF]	NHS revenue → AIM IPO 2020 (~£26M) [PitchBook]	KFF ethics investigation (official flown to London), slow Soluna uptake → share-price plunge; one-state concentration risk [RNS]
eQuoo PsycApps · narrative RPG for emotional fitness · SUB-SCALE	Resilience + depression/anxiety scales; 5-week RCT in students, JMIR Mental Health 2023; notably low attrition [JMIR]	UK (UCL co-authors) + NHS Apps Library listing (2019) → credential leveraged for US schools/colleges expansion raise [TC]	£1.5M seed (Morningside) — proof that a published RCT is a seed-scale problem [UKTN]	Founder-authored evidence (COI), self-report only, 5-week follow-up; NHS Apps Library later wound down, weakening the badge
Betwixt interactive-fiction mental-health game · EMBRYONIC	Unusually rigorous battery for an indie: DERS-SF (primary), PHQ-9, GAD-7, SCS-SF, SWEMWBS [trial record]	UK — University of Nottingham + Lincolnshire NHS Trust study (n=8, completed 2025); English-language global app stores	Bootstrapped; no disclosed rounds [CB Insights]	n=8 single-case evidence; founder's own essay concedes the category's engagement model is broken [essay]

SuperBetter gamified resilience (McGonigal) · STALLED	CES-D / GAD-7 in Penn RCT (n=283, 2015); OSU pediatric concussion pilot (NIH-funded) [PubMed]	US academic trials → global consumer + schools/health systems (Parkland Health); Zambia pilot with Children for Health [Republic]	~\$270K lifetime (\$170K F&F + \$100K crowdfunding) — Kickstarter-scale	Venture entity wound down mid-2010s; flagship evidence is one small, dated, high-dropout RCT
Zombies, Run! Six to Start · narrative fitness game · NEAR-DEATH, RESCUED	Engagement/behavioral metrics only; UCL qualitative study followed the hit [PubMed]	UK Kickstarter (2012) → global app stores day one; story content language-bound (localization ceiling)	Crowdfunded build; whole studio sold for \$9.5M (OliveX/Animoca 2021) [TheGamer]	Content-treadmill economics; crypto-owner misalignment cut team to 2, co-creator bought it back (Nov 2025) [Techbuzz]
Lumosity brain training · FTC-FINED	Proprietary in-game indices (LPI) — exactly what regulators rejected as evidence	US claims → 85M global users; 2014 Stanford + Max Planck consensus (~70 scientists) against brain-game transfer claims [SciAm]	~\$70M (Discovery Communications, Menlo)	\$2M FTC settlement (2016) — \$50M judgment suspended; category-defining wording rules: health claims need RCT-grade evidence [FTC]
Happify / Twill gamified positive psychology · FIRE-SALE	Subjective wellbeing score + symptom scales; real-world observational data (self-selected users — acknowledged limitation) [PMC]	US consumer → US enterprise/pharma co-promotions; no regulatory pathway completed	~\$118M raised	Sold to DarioHealth for \$22.4M (Feb 2024) — >80% value destruction [STAT]
Headspace meditation → care platform · B2B RETREAT	PSS/burnout self-report in partner studies (UCSF); solicited external researchers because cognitive evidence was inconclusive [Fierce]	UK-founded → LA → 70+ countries via app stores (marketing-led transfer, no regulatory route); Ginger merger bolted on clinical care	~\$215M + Ginger ~\$220M; merged at ~\$3B (2021) [HC Dive]	Three layoff rounds 2022–24, therapist network gutted, retreat to employer channel [BHB]
Calm + Finch + Fabulous consumer wellness tier · ALIVE, EVIDENCE-LIGHT	Calm: PSS/PROMIS in self-funded ASU trials (lead author became Calm's science head) [JMIR] ; Finch & Fabulous: no clinical instruments, engagement only	US → global app stores; Fabulous transfers Duke behavioral-lab design principles, not evidence [Fabulous] ; Finch grew via organic TikTok	Calm ~\$218M at \$2B (Lightspeed, Goldman, TPG) [TC] ; Finch/Fabulous funding not public	Calm: 20% layoffs 2022, science team shake-up [STAT] ; Finch: zero clinical evidence, cosmetics-only monetization; Fabulous: paywall/cancellation complaints [critique]

TABLE 2 Failures, lawsuits & regulatory actions

Who got punished, for what, and for how much — the case law of this category.

CASE	WHAT HAPPENED	NUMBERS & DATES	LESSON FOR CONTENT THERAPY
Lumosity — FTC	Deceptive brain-training claims (work/school performance, cognitive decline) without RCT-grade evidence; undisclosed incentivized testimonials [FTC case]	\$2M paid (of a \$50M judgment), Jan 2016	Outcome claims require randomized, controlled, qualified-researcher evidence. "Fun" language is free; "improves" language is regulated.
BetterHelp — FTC	Shared intake health data with Facebook/Snapchat/Criteo/Pinterest for ad retargeting after promising privacy; misleading HIPAA seal [FTC]	\$7.8M refunds; permanent ad-use ban; ~800K refund notices (2024)	Zero ad pixels/SDKs on any surface touching intake or symptoms. FTC Section 5 is de facto HIPAA for wellness apps.
Cerebral — DOJ/FTC/NY AG	Prescribing-pressure culture (DEA/DOJ probe), health-data sharing with ad platforms (3.2M users), cancellation dark patterns [DOJ]	~\$7M FTC (2024) + \$740K NY AG (2023) + \$3.6M+ DOJ (2024)	One-click cancellation, no trackers, no growth metrics tied to clinical decisions. Founder liability is now personal.
Replika — Italian Garante	Emotional-AI companion with no age gating, no GDPR legal basis; ban → panic feature removal became its own harm event [EDPB]	Ban Feb 2023; €5M fine May 2025 + LLM training-data probe	EU launch blockers: age verification + documented legal basis. Never let users attach to features compliance may remove.
NEDA "Tessa"	Vendor silently added a generative layer to a scripted wellness bot; it gave dieting advice to eating-disorder patients; pulled Jun 2023 [NPR]	No fine — reputational destruction	Contractual change control on generative components; red-team against your most vulnerable user, not the average one.
Koko	~30,000 GPT-3-assisted support messages to ~4,000 users without informed consent (2023) [NBC]	No fine — ethics firestorm	Disclose AI involvement before the interaction; consent + documentation for anything experiment-shaped.
Character.AI / Google	Wrongful-death suits over teen users; First Amendment defense <i>rejected</i> May 2025 — chatbot output ruled a product; under-18 open chat banned Nov 2025; suits settled Jan 2026 [CNN]	Settlement terms confidential; FTC 6(b) inquiry Sept 2025; CA SB 243	The whole "app that talks about feelings" category now carries product-liability exposure: age assurance, crisis detection, no dependency-cultivating patterns — designed in and documented.
Babylon Health	Overclaimed AI triage + unprofitable value-based care; \$4.2B SPAC → insolvency Aug 2023, sold for parts [TC]	~\$1.2B raised; ~99% equity wipeout	AI accuracy claims attract expert adversaries; NHS-style contracts confer legitimacy, not margin.

§ 3 Failure taxonomy — ranked by lethality

- 01 Reimbursement gap (kills outright). FDA clearance does not create a payer. Pear (\$1.6B → \$6M), Akili (\$1B → \$34M), Woebot (quit), Babylon. → Clearance is armor, never the revenue plan.
- 02 Consumer-subscription unit economics (grinds down). CAC + churn beat even the category leaders — Calm and Headspace both retreated to employer channels. → Build B2B distribution early; model churn honestly.
- 03 Claims overreach (FTC's oldest playbook). Lumosity's \$2M order wrote the category's wording rules. → Map every public claim to an evidence tier.
- 04 Privacy monetization (now reliably punished). BetterHelp, Cerebral, Replika — combined >\$20M and permanent bans. → Zero third-party trackers, ever.
- 05 AI safety incidents (escalated from PR to product liability). Tessa, Koko, Replika, Character.AI. → Human-authored clinical content, hard-coded crisis paths, vendor change control, age assurance.

The most dangerous combination — clinical-sounding claims + consumer ad-tech growth stack + generative AI aimed at vulnerable users — triggers all five modes at once. The two safest observed positions: pure wellness with no outcome claims and no trackers, or evidence-backed products with deliberately modest claims.

§ 4 How research transfers across markets — observed routes

UK institutional route **PROVEN**

Oxford/UCL evidence → NICE guidance or NHS credential → US employer/PBM/government dollars. Big Health (NICE 2022 → Evernorth/CVS), Kooth (NHS data → California \$188M), eQuoo (NHS Apps Library → US schools raise).

India → global **RARE**

Wysa: Bangalore build → UK NHS deployments → US FDA Breakthrough. Anonymity-beats-stigma hypothesis proved portable; English-language core did the traveling.

US Rx route **GRAVEYARD**

US RCT → FDA clearance → prescription + payer market. Every company that relied on it (Pear, Akili) died or fire-sold. CMS G-codes (2025–26) may finally change this — unproven.

Marketing-led transfer **EXPENSIVE**

App-store globalization with no regulatory anchor (Headspace, Calm, Fabulous, Finch). Works while paid CAC works; post-COVID it stopped working for the leaders. Nobody in the set used Germany's DiGA as a springboard.

§ 5 Cost benchmarks

~£1.5–2M

Seed-scale gamified product + published RCT + NHS listing (eQuoo). The realistic evidence budget for Content Therapy's clinical foundation.

\$270K–\$9.5M

The indie band: SuperBetter's lifetime funding (\$270K); a mature 10M-user narrative fitness game changed hands for \$9.5M (Zombies, Run!).

\$150–300M+

A regulated prescription DTx through pivotal RCT and commercialization (Akili, Pear) — with, so far, zero examples of the bet paying back.

\$34M / \$22M / \$6M

Actual disclosed exits of the evidence-first cohort (Akili / Twill / Pear) vs. \$2–3B private peaks (Calm, Headspace) that never exited.

TABLE 3 Content Therapy pre-mortem — full risk register

Method: Gary Klein's pre-mortem — "It is March 2028 and the project is dead; write the story of how." Each story is mapped to a documented precedent, an early-warning signal, and the cheapest test that defuses it.

#	FAILURE STORY	LIKELIHOOD	IMPACT	EARLY-WARNING SIGNAL	MITIGATION + CHEAPEST TEST	PRECEDENT
R1	We bet the company on reimbursement; clearance arrived, coverage didn't, runway died in the 18–36-month gap	HIGH	Fatal	Clinical spend growing faster than non-payer revenue	Entertainment + employer revenue carries the company; clearance is optional armor. Test: pre-sell pilot ARG at \$50–75K, months 0–3	Pear, Akili
R2	Narrative entertained avoidance — retention was great, users stopped doing real-world tasks, clinical endpoints failed	MED	Fatal	Inverse correlation between narrative satisfaction and task completion in pilots	Behavioral gate before clinical spend: n=20, ≥60% complete a real-world micro-task within 48h	Documented AI-therapy avoidance critique
R3	Onboarding lost to ChatGPT — profiling took 2 minutes, the \$0 default took zero	HIGH	Severe	Funnel abort spikes at profiling steps	Hard 90-second ceiling; A/B "play immediately" vs full intake, n=200 landing-page visitors	ChatGPT+Notes DIY benchmark
R4	Claims overreach — "EMDR encoded in cinema" / "treats migraine" drew FTC letters and clinical-community backlash	MED	Severe	Marketing copy drifting ahead of the evidence tier map	Kravitz language rules adopted verbatim ("EMDR-informed... supervised pilot"); claims-evidence audit each release	Lumosity \$2M

R5	AI safety incident — generated content harmed a vulnerable user; discovery surfaced no documented safety design	MED	Fatal	Any unreviewed generative path to a clinical-population user	Human-authored clinical content only; hard-coded crisis escalation; vendor change-control clauses; age assurance; document everything	NEDA Tessa; Character.AI (output = product, May 2025)
R6	Consumer economics ground us down — CAC rose, churn stayed, subscriptions never covered content	HIGH	Severe	CAC payback >12 months; post-trial churn >20%/mo	B2B (employers/EAPs \$50–100/user/yr) from month 6; D2C as brand-building; honest churn model	Calm –20%, Headspace ×3 layoffs
R7	ARG bridge never materialized — no case studies, studios wouldn't pre-pay, seed carried everything	MED	Severe	5 studio pitches fail by month 4	Pre-sell at \$50–75K before engine completion; deprioritize surface if month-4 gate fails	No public ARG fee benchmarks (unverified WTP)
R8	Privacy monetization reflex — a growth hire added pixels to intake pages	LOW	Severe	Any third-party SDK in the app build	Zero-tracker policy on authenticated surfaces, enforced by build audit	BetterHelp \$7.8M; Cerebral ~\$11M total
R9	Content treadmill — per-narrative production cost outran revenue	MED	Moderate	Cost per shipped scene not falling quarter over quarter	Parameterized template reuse (120-scene modular library); ARG content is studio-supplied IP	Zombies, Run! near-death
R10	Evidence credibility discount — investors/clinicians found founder-authored studies and overstated science in our own materials	MED	Moderate	Draft claims failing external fact-check (it already happened: market size, therapy cost, 432 Hz)	Independent PIs for all studies; this dossier's corrections shipped <i>with</i> the pitch; 432 Hz/binaural framed as design choice, not evidence	Category-wide COI pattern (Espie, Huberty, Litvin)
R11	LLM commoditization — frontier chatbots felt better than our scripted safety; users benchmarked us against free	HIGH	Severe	Session frequency dropping as frontier model releases land	Compete on device-native screenlife + physician trust, never on conversation quality; ship fast	Woebot exit, Jun 2025
R12	FDA flagged generative content — pre-sub feedback added 12+ months to the clinical path	MED	Severe	Pre-sub questions about content provenance	Template-first architecture already confines LLM to consumer surface; clinical loop is human-authored by design	FDA DMHT guidance posture

R13	Astrology onboarding poisoned clinical credibility — advisory board and healthcare investors balked	MED	Moderate	Advisory-board pushback; diligence questions	Separate consumer brand from clinical entity (names, databases, marketing); board veto over consumer features; clinical-first investor narrative	Cerebral reputational spiral
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Bottom line for the pitch: the graveyard is prescription-first; the survivors sell to consumers, employers and governments while keeping claims one notch below their evidence. Content Therapy's three-surface design is the correct shape — provided the R1/R2/R3 gates are treated as real kill-criteria, not slideware.

Compiled with the VoC market-research protocol: parallel research streams, primary-source links for every claim, mandatory documented weaknesses, "not public" where undisclosed. Some user-complaint quotes are secondary-sourced via press coverage — spot-check before high-stakes use. Companion documents: pitch deck artifact, data-verification report and full research files in the shared Google Drive folder (Content Therapy). Compiled 2026-09-01 for Ann.